

03 — Medical Safety & Knowledge Rules

This is a safety/knowledge boundary document. Coding agents must not invent medical rules.

Core Principle

- The app is an educational tracker, not a medical device, diagnostic tool or substitute for a qualified clinician.
- All medical claims must be traceable to a named source and review date.
- When evidence is uncertain, the UI must say estimate, may, can or varies rather than presenting certainty.

Fertility Window

- ASRM describes the fertile window as the 6-day interval ending on the day of ovulation.
- Calendar-based ovulation prediction can be inaccurate because ovulation timing varies between cycles, including among people with regular cycles.
- Therefore the app must label calendar-derived fertile days as estimated/predicted, not confirmed.
- Optional ovulation-test and cervical-mucus observations can be recorded as user observations. They should not be converted into diagnosis.

Trying to Conceive

- ASRM reports that intercourse every 1–2 days during the fertile window can maximize fecundability, while less frequent intercourse can still be effective.
- Do not create stressful mandatory schedules. Use neutral language such as 'Estimated fertile window' and 'Many couples choose to have intercourse every 1–2 days during this window.'
- Do not claim any sexual position, post-intercourse routine, supplement, food or timing trick guarantees pregnancy.

Folic Acid

- ASRM guidance states that women wishing to become pregnant should take daily folic acid supplementation of 400 micrograms to reduce neural-tube-defect risk.
- Do not prescribe personalized supplementation. The app may show general educational guidance and tell users to discuss individual needs with a clinician.

Infertility Escalation

- ASRM's 2023 definition/guidance states that evaluation should generally be initiated after 12 months of regular unprotected intercourse when the female partner is under 35, and after 6 months when she is 35 or older, with earlier evaluation when history or findings warrant it.
- The app must not diagnose infertility. It can show a neutral reminder to consider medical evaluation based on time trying and age, with a clinician disclaimer.

Pregnancy Safety

- Pregnancy symptom content must distinguish common symptoms from symptoms that may need urgent assessment.
- If a user reports potentially serious symptoms, do not diagnose. Provide a clear recommendation to seek appropriate medical care and emergency help when the situation may be life-threatening.

- Country-specific emergency numbers should not be hard-coded globally. Make emergency guidance configurable by country.

Forbidden Medical Behavior

- Do not diagnose from symptoms.
- Do not recommend stopping or changing prescription medicine.
- Do not calculate fetal health from symptoms.
- Do not claim pregnancy viability from a single symptom or home test.
- Do not tell a user that everything is normal when serious symptoms are present.
- Do not use AI-generated medical text without a safety prompt, source grounding and human-reviewed content for production.

Primary Sources

- Medical content in this document is product guidance, not a diagnosis or treatment protocol. Key fertility guidance references: American Society for Reproductive Medicine (ASRM), Optimizing Natural Fertility (2022), and ASRM Definition of Infertility (2023). Pregnancy urgent-symptom concepts should be aligned with current local clinical guidance. Sources: <https://www.asrm.org/practice-guidance/practice-committee-documents/optimizing-natural-fertility-a-committee-opinion-2021/> ; <https://www.asrm.org/practice-guidance/practice-committee-documents/definition-of-infertility/>